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# Mental Health Diversity and Integrative Recovery: A Lived-Experience Case Study of Voluntary, Trauma-Informed, and Culturally Responsive Care

**Dr. Letitia Antoinette**<sup>1</sup>

**Dr. Daniel Kapuscinski**<sup>2</sup>

**LaVada S. England**<sup>3</sup>

1. Founder/President, Letitia Antoinette World University

2. Registrar General, Awedu Institute of Business Management and Leadership

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**\*Corresponding Author :Dr. Letitia Antoinette**

**Email: [info@letitiaantoinette.co.uk](mailto:info@letitiaantoinette.co.uk)**

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## Abstract :

This study examines mental health diversity through an in-depth qualitative lived experience case study exploring the intersections of trauma, neurodivergence, chronic physical illness, gender, ethnicity, and societal perception within long term recovery processes. Grounded in trauma informed & recovery oriented paradigms, the research employs a narrative inquiry methodology informed by Elisabeth Kübler-Ross's Grief Model and a customised SMART-SWOT reflective framework developed in Restored (Kapuscinska, 2024). Data sources include longitudinal reflective narratives, contextual clinical observation, arts-based materials drawn from the award-winning documentary *Bipolar: What They Don't Tell You* and culturally responsive psychosocial support practices. All mental health interventions & hospital admissions described in this study were voluntary & undertaken through informed, person-centred decision making. Findings indicate that sustained psychological stability and reduced crisis frequency were most effectively supported through an integrative mental health approach combining selective Western pharmacological care, prescribed medicinal cannabis, meaningful daily occupation, creative practice, and cultural spiritual support within a trusted multidisciplinary network. This study contributes to interdisciplinary mental health scholarship by challenging reductive diagnostic models and emphasising the centrality of patient autonomy, environmental compatibility, cultural responsiveness, and integrative care in long term recovery. By foregrounding lived experience as an epistemological resource, the research advances trauma informed, voluntary, and person-centred approaches to mental health practice, policy, and knowledge production.

**Keywords :** Mental health diversity; lived-experience research; trauma-informed care; voluntary treatment; integrative recovery; recovery-oriented practice; patient autonomy; cultural responsiveness; narrative inquiry; prescribed medicinal cannabis

## Introduction:

Mental health diversity encompasses a wide range of psychological, neurological, physical, social, and cultural experiences that shape how individuals understand themselves, interact with others, & navigate daily life. Although contemporary mental health discourse increasingly acknowledges trauma-informed and person-centred approaches, dominant biomedical models often remain limited in their capacity to account for the cumulative and intersecting effects of neurodivergence, chronic illness, ethnicity, gender

lived trauma, and environmental compatibility. These limitations are particularly evident in the experiences of women from ethnically diverse backgrounds, whose narratives frequently fall outside standardised diagnostic frameworks. Lived-experience research has demonstrated that misinterpretation, over-pathologisation & insufficient cultural responsiveness may exacerbate distress rather than facilitate recovery. This study examines mental health diversity within the context of an interracial family of four (three males and one female, aged 45, 35, 14, and 10) of British, Polish, Caribbean, and African heritage.

The analysis focuses primarily on the mother, who has received diagnoses including bipolar disorder, functional neurological disorder (FND), autism, attention deficit hyperactivity disorder (ADHD), dyslexia, fibromyalgia, temporomandibular joint dysfunction, tarsal coalition, and post-traumatic stress disorder (PTSD). Two children were also diagnosed with autism, one later in childhood, highlighting intergenerational & developmental dimensions of neurodivergence. The participant's initial psychiatric diagnosis followed her second pregnancy & natural delivery. Pharmacological intervention for suspected postnatal depression preceded an episode of medication associated psychosis, resulting in voluntary hospital admission. This experience served as a catalyst for critical reflection on diagnosis, medication effects, autonomy, and the role of environment in mental health. Over time, the participant actively pursued voluntary, integrative, culturally responsive strategies aimed at preventing crisis escalation and reducing reliance on inpatient care. These experiences informed the development of the Restored framework, which integrates reflective practice, environmental awareness & personalised self-management.

### **The objectives of this study are to:**

1. To explore how mental health diversity emerges through intersecting diagnoses, trauma, and identity.
2. To examine the impact of societal perception and stigma on self-identity and recovery.
3. To evaluate voluntary, integrative mental health approaches in sustaining long-term psychological stability.
4. To contribute to trauma-informed, recovery-oriented mental health practice and policy.

### **Literature Review:**

Over recent decades, mental health care has undergone a substantive shift from predominantly clinical, pathology oriented models toward more holistic and integrative approaches that recognise the significance of diversity, lived experience, cultural context, and trauma-informed practice in recovery processes. This evolution has emerged in response to sustained critique of traditional biomedical frameworks, which have been shown to inadequately capture the complexity of individual experience, particularly among historically marginalised & minoritised populations (Browne et al., 2016; Herman, 1997). In contrast, integrative recovery models emphasise collaboration, cultural respect, and empowerment through voluntary participation in care, reframing recovery as a personalised and socially situated process rather than solely a clinical outcome (Topor et al., 2011).

Contemporary literature increasingly positions voluntary, trauma-informed, and culturally responsive care as central to ethical and effective mental health practice. Within this body of work, lived experience is recognised not only as an outcome of recovery but also as a critical source of knowledge that enhances understanding of recovery dynamics, service engagement, and long-term wellbeing.

### **Mental Health Diversity and Recovery Frameworks:**

The concept of mental health diversity foregrounds the existence of multiple, context-dependent pathways to recovery shaped by cultural, social, economic, and personal identities (Sue et al., 2012). Historically, mental health services have largely been developed within Western epistemological traditions that prioritise biomedical explanations and standardised treatment protocols (Kleinman, 1988). While these approaches have contributed to advances in diagnosis and treatment, scholars argue that they often marginalise individuals from non-Western, Indigenous, and racially minoritised backgrounds whose explanatory models of distress and healing diverge from dominant paradigms (Gone, 2013). Empirical research highlights the role of structural inequality, cultural disconnection, & systemic discrimination in exacerbating mental health disparities (Williams & Mohammed, 2009). These findings suggest that interventions grounded in cultural meaning systems & community-based values may enhance both engagement and outcomes. As such, recovery-oriented frameworks increasingly call for mental health services that are flexible, pluralistic, and responsive to diverse lived realities. Recovery models informed by lived experience, including the Personal Recovery paradigm, shift the focus from symptom reduction toward meaningful life participation, self-determination, and resilience (Anthony, 1993; Leamy et al., 2011). These models challenge clinician-led definitions of recovery by privileging the narratives and priorities of individuals with lived experience. Central to this philosophy is the alignment with voluntary care, which emphasises autonomy, shared decision-making, and consumer choice in the development of care plans (Ridgway, 2001). Evidence suggests that when individuals are positioned as active partners in their care, they report higher satisfaction, enhanced self-efficacy, and improved long-term outcomes (Slade et al., 2014).

### **Trauma-Informed Care and Its Relevance:**

Trauma exposure is highly prevalent among individuals accessing mental health services, and there is growing recognition that traditional treatment environments may inadvertently retraumatise service users when trauma histories are overlooked or minimised (Sweeney et al., 2016).

Trauma informed care (TIC) has emerged as a response to this concern, advocating for the systematic integration of trauma knowledge into all aspects of service delivery (Fallot & Harris, 2009). Core principles of TIC include safety, trustworthiness, choice, collaboration, and empowerment (SAMHSA, 2014), which closely align with voluntary and recovery oriented models of care. A growing body of evidence indicates that trauma-informed approaches are associated with improved engagement, reduced use of coercive practices, and decreased risk of retraumatisation across diverse settings, including inpatient psychiatric services, community mental health programs, and substance use recovery contexts (Isobel & Edwards, 2015). Importantly, TIC is increasingly conceptualised not merely as an individual clinician competency but as an organisational framework requiring systemic change to policies, environments, and power structures (Menschner & Maul, 2016). As such, trauma-informed practice is now widely regarded as foundational to ethical, person centred mental health care.

### **Culturally Responsive Care: Beyond Cultural Competence:**

Culturally responsive care extends beyond traditional notions of cultural competence to encompass sustained, reflexive engagement with clients' cultural identities, values, communication styles, and worldviews (Sue, 2006). While cultural competence has been influential in raising awareness of cultural difference, it has been critiqued for treating culture as static, essentialised, or reducible to discrete knowledge sets (Kirmayer, 2012). In contrast, cultural responsiveness emphasises dynamic, relational, and contextually grounded practices that acknowledge culture as fluid and co-constructed within therapeutic encounters. The literature demonstrates that culturally adapted interventions are associated with improved access, engagement, and clinical outcomes among racially and ethnically diverse populations (Bernal et al., 2009). Community-based mental health programs that incorporate culturally meaningful practices such as storytelling, spiritual traditions, collective healing, and family involvement have been shown to foster stronger therapeutic alliances and greater continuity of care than conventional models alone (Gone, 2009). From this perspective, culture is not peripheral to recovery but fundamentally shapes the expression of distress, the meaning of healing, and the pathways through which recovery is realised (Tseng, 2004).

### **Lived Experience in Integrative Recovery:**

Lived experience perspectives contribute critical insights into recovery processes by foregrounding agency, narrative

identity, relationality, and peer support (Mead & MacNeil, 2006). The inclusion of peer-led services and service-user involvement in program design has become a defining feature of recovery-oriented mental health systems (Davidson et al., 2006). Qualitative studies consistently highlight the importance of trusting therapeutic relationships, flexible and responsive service models, & recognition of individual and cultural strengths in supporting recovery (Repper & Carter, 2011). Case study methodologies within lived-experience research further illuminate the interaction between personal meaning-making processes and formal care structures. Such studies demonstrate that integrative approaches when voluntary, trauma informed, & culturally attuned can enhance self efficacy, support identity reconstruction & reduce reliance on crisis-driven interventions (Topor et al., 2011). This body of work positions lived experience not merely as a supplementary perspective but as an epistemological foundation for intervention design, service evaluation, and mental health policy development.

### **Synthesis and Gaps:**

Collectively, the literature converges on the conclusion that effective mental health care must integrate diversity, trauma awareness, and cultural responsiveness within voluntary, recovery-oriented frameworks that centre lived experience. Despite this consensus, significant gaps remain. Many mental health systems continue to be structured around clinician driven pathology focused models that insufficiently account for personhood, context, and agency. Additionally, the integration of Indigenous and non-Western healing traditions within mainstream services remains limited, often constrained by institutional barriers and epistemic inequities (Kirmayer et al., 2011). Furthermore, while integrative recovery models are increasingly advocated, robust longitudinal and comparative evaluation studies remain relatively scarce. There is a pressing need for methodologically rigorous research that examines long-term outcomes of voluntary, trauma-informed, and culturally responsive approaches across diverse populations & service contexts. In summary, contemporary mental health scholarship reflects a paradigmatic shift toward holistic, person-centred care that is voluntary, trauma-informed, and culturally responsive. Such approaches are essential for honouring mental health diversity and supporting authentic recovery pathways as articulated by individuals with lived experience. An integrative framework does not reject clinical expertise but situates it within broader ethical, relational, and cultural contexts that prioritise dignity, agency, and meaning. Ongoing research, policy innovation, and service redesign are required to realise mental health systems that are equitable, inclusive, and genuinely recovery oriented.

## Methodology:

This study adopts a qualitative, reflective case study methodology grounded in lived-experience research and narrative inquiry. The approach prioritises depth, reflexivity & contextual understanding rather than generalisability. Data were generated through longitudinal self-reflection, lived narrative documentation, & contextual analysis of clinical and community experiences. Analytical frameworks included Kübler-Ross's Grief Model and a customised SMART SWOT framework developed in Restored (Kapuscin ska, 2024), supporting structured reflection on emotional adaptation, strengths, vulnerabilities, opportunities, risks, and environmental compatibility. The award-winning short documentary *Bipolar: What They Don't Tell You* is included as an arts-based and educational artefact supporting public understanding and stigma analysis. It is not treated as empirical clinical evidence but as a narrative and reflective contribution consistent with arts based research methodologies.

## Ethical Framework:

This study adheres to UK ethical standards for qualitative & lived experience research, including respect for autonomy, dignity, confidentiality & informed consent. All treatments, cultural practices, and hospital admissions described were voluntary and initiated by the participant as part of an informed self management & safety strategy. No compulsory or coercive interventions were used. Informed consent was obtained and revisited throughout the research process. Identifying information relating to family members and children has been anonymised. No clinical records were accessed; all data derive from first person narrative and reflective documentation.

## Positionality and Reflexivity Statement:

This research is situated within a lived-experience and scholar practitioner paradigm, wherein the author occupies the dual role of researcher and primary participant. This positionality is acknowledged transparently in line with best practice in qualitative, narrative, and trauma informed research, where reflexivity is recognised as a methodological strength rather than a limitation. As an author, researcher, and individual with lived experience of complex mental health diversity, neurodivergence, chronic physical illness, and trauma, Kapuscinska brings experiential knowledge that informs both the framing and interpretation of the data. This insider perspective enabled nuanced understanding of embodied experience, identity negotiation, and recovery processes that may be inaccessible through detached observational methods alone.

Reflexive practice was employed throughout the research process to mitigate potential bias, including structured reflection using the SMART SWOT framework, critical engagement with existing literature & deliberate distinction between descriptive narrative and analytical interpretation. The study does not claim objectivity in the positivist sense but aligns with interpretivist and constructivist traditions that value situated knowledge. The author's positionality also informed a strong ethical commitment to voluntary participation, patient autonomy, and non-coercive care, shaped by prior experiences within mental health systems.

This informed the study's emphasis on trauma-informed care, recovery oriented practice, and culturally responsive support frameworks. Acknowledging power dynamics between clinician, researcher, and participant was central to maintaining ethical integrity. Importantly, while the author is also the creator of the Restored framework, this self-citation is presented transparently as a methodological contribution grounded in lived experience rather than as an attempt to universalise findings. The framework is offered as a reflective tool that may inform future inquiry, not as a prescriptive model. By foregrounding reflexivity, this research seeks to contribute responsibly to mental health scholarship, demonstrating how experiential knowledge can coexist with academic rigour, ethical accountability, and interdisciplinary relevance.

## Results:

### Theme 1: Voluntary Admission as Insight Led Self-Care:

Voluntary hospital admission functioned as a preventative, insight led strategy rather than a loss of autonomy. Over time, enhanced self regulation corresponded with reduced reliance on inpatient care.

### Theme 2: Integrative Mental Health and Medicinal Cannabis:

Prescribed medicinal cannabis, used voluntarily and under medical supervision, supported symptom regulation related to chronic pain, sleep disturbance, neurological distress, & trauma related dysregulation. Lived experience accounts indicated improved tolerability relative to previous pharmacological treatments.

### Theme 3: Cultural and Spiritual Support:

Engagement with shamanic practitioners formed part of a culturally responsive psychosocial support system, contributing to emotional grounding, meaning making & resilience without replacing clinical care.

#### **Theme 4: Meaningful Occupation and Creative Restoration:**

Re-engagement with singing, movement, tai chi, and creative authorship was associated with improved emotional regulation, agency, and identity coherence.

#### **Theme 5: Arts-Based Knowledge Translation:**

The documentary *Bipolar: What They Don't Tell You* contributed to stigma reduction, public education, & validation of lived experience beyond clinical contexts. This study contributes to contemporary discourse on mental health diversity by demonstrating how recovery and long-term stability can be sustained through voluntary, trauma-informed, and integrative mental health practices grounded in lived experience research. Rather than attributing improvement to a single intervention, the findings highlight the importance of patient autonomy, cultural responsiveness, & meaningful occupation within recovery-oriented practice.

#### **Discussion:**

##### **Mental Health Diversity and the Limits of Diagnostic Reductionism:**

The findings reinforce critiques of diagnostic reductionism, demonstrating how co-occurring mental health conditions, neurodivergence, chronic illness, and trauma interact dynamically rather than as discrete clinical entities. Stability emerged through integrative, identity-affirming approaches rather than intensified diagnostic management.

##### **Voluntary Treatment, Autonomy, and Recovery Oriented Practice:**

Voluntary engagement, including hospital admission, functioned as a stabilising mechanism. Autonomy, choice, and collaboration aligned closely with trauma informed and recovery oriented frameworks, supporting sustained engagement and reduced crisis escalation.

##### **Integrative Mental Health & Prescribed Medicinal Cannabis:**

Medicinal cannabis functioned as a supportive, supervised intervention within a broader integrative framework. While not generalisable, the findings highlight the importance of informed consent, patient led decision making, and multi-disciplinary oversight.

##### **Cultural Responsiveness and Shamanic Support:**

Cultural spiritual practices contributed meaningfully to emotional regulation and identity coherence. Respectful integration strengthened engagement without compromising clinical safety.

#### **Meaningful Occupation, Creativity, and Identity Restoration:**

Meaningful occupation and creative practice emerged as central mechanisms of recovery, supporting emotional regulation and identity restoration in alignment with occupational and recovery oriented literature.

#### **Arts-Based Evidence and Knowledge Translation:**

Arts based outputs functioned as valid knowledge translation tools, extending mental health discourse beyond clinical and academic boundaries.

#### **Implications for Practice, Policy, and Research:**

##### **Findings suggest the need to:**

- Expand voluntary, autonomy supportive pathways in mental health services.
- Ethically evaluate integrative approaches rather than dismiss them a priori.
- Incorporate cultural and spiritual supports within multi disciplinary care.
- Prioritise environment, occupation, & identity alongside diagnosis.

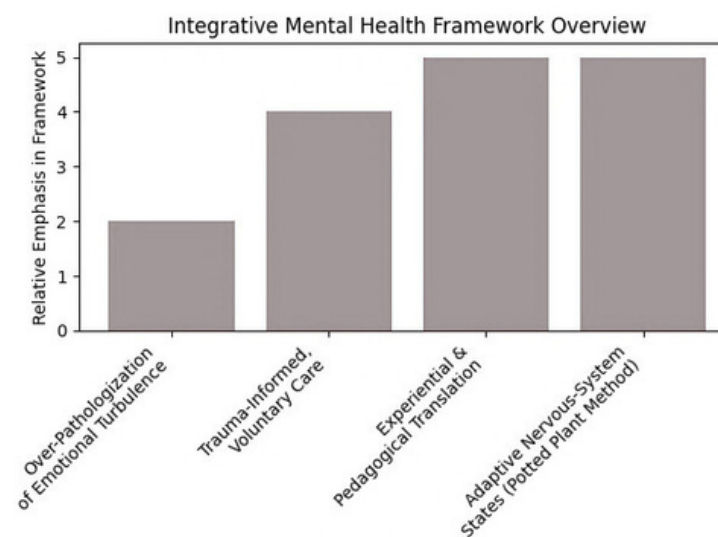
Future research may build on this work through participatory, comparative, and interdisciplinary designs.

#### **The Potted Plant Method as a Neuro Emotional Regulation Model:**

##### **A Lived Experience Contribution to Integrative Mental Health Scholarship**

Mental health discourse has historically struggled to distinguish normative emotional dysregulation from clinically significant psychiatric pathology. This conceptual ambiguity has, at times, contributed to the over pathologisation of adaptive responses to trauma, chronic stress exposure, and unmet developmental needs (Herman, 1992; van der Kolk, 2014). Contemporary trauma and developmental research increasingly emphasises that many behavioural & affective disturbances reflect nervous system adaptations to environmental threat rather than primary disease processes (Porges, 2011; Schore, 2003). While the doctoral research of Duchess Dr. Letitia Kapuscinska provides an empirical framework for trauma-informed, voluntary, and integrative mental health care, the present contribution offers an experiential and pedagogical translation model designed to support emotional insight, self regulation, and participatory recovery. The Potted Plant Method is introduced as a trauma informed, neuro emotional mapping framework that

renders complex nervous-system and attachment processes accessible through visual and developmental metaphor. Rather than construing cyclical emotional states as inherently pathological, the framework conceptualises fluctuations such as depression, frustration, inspiration, & resilience as adaptive autonomic responses to relational & environmental conditions. This position is consistent with contemporary models of autonomic regulation and stress responsivity (Porges, 2011; Siegel, 2012)



## Emotional Turbulence vs Mental Illness:

### All human beings experience:

- Emotional variability
- Intrusive or repetitive cognition
- Stress related dysregulation
- Cycles of depletion and restoration

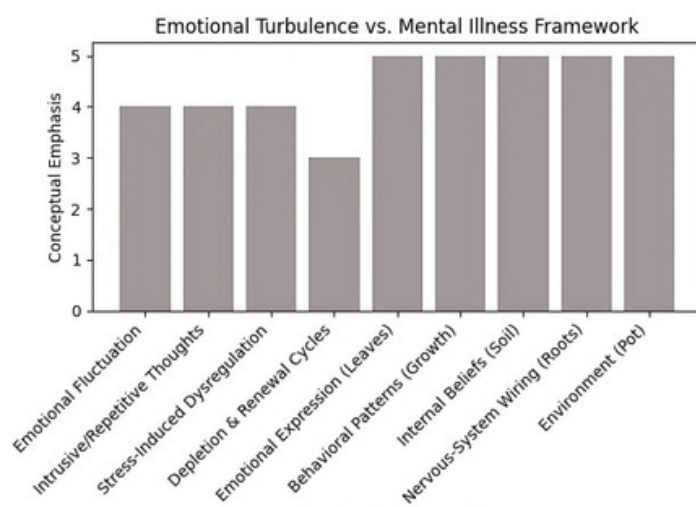
From a neurodevelopmental perspective, these processes represent adaptive regulatory attempts rather than diagnostic indicators in isolation (Schoore, 2003; Siegel, 2012). Severe psychiatric conditions such as bipolar spectrum disorders, psychotic disorders, or neurological dysregulation require clinical diagnosis & may necessitate pharmacological stabilisation. However, research in trauma psychology suggests that many individuals presenting for mental health care exhibit unresolved trauma responses, attachment disruption, or chronic stress activation rather than primary psychiatric disease (Herman, 1992; van der Kolk, 2014).

### The Potted Plant Method conceptualises psychological functioning as the interaction of multiple regulatory layers:

- Emotional expression

- Behavioural adaptation
- Cognitive belief structures
- Nervous system organisation
- Environmental containment

This systems based perspective aligns with biopsychosocial and interpersonal neurobiology models, which emphasise the integration of physiological regulation, relational experience, and meaning-making processes (Engel, 1977; Siegel, 2012).



## A Neuro Developmental & Trauma Informed Framework:

The framework maps primary autonomic regulatory states onto cyclical seasonal metaphors:

- Winter — immobilisation or depressive states associated with freeze responses
- Summer — mobilisation and frustration associated with fight activation
- Spring — regulated expansion and exploratory engagement
- Autumn — integrated stability and adaptive resilience

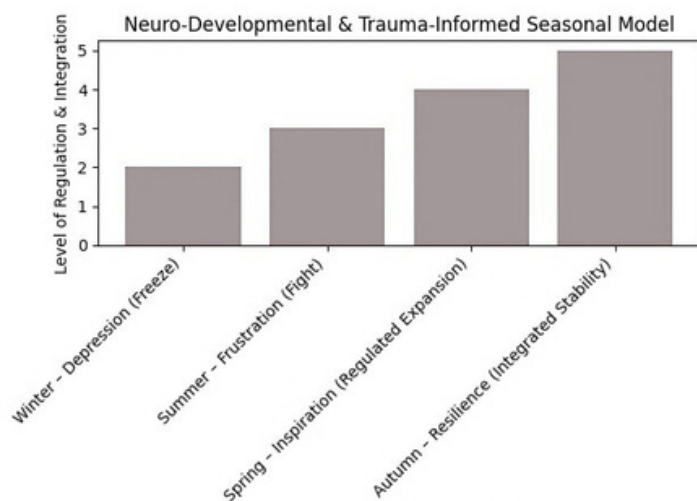
These cyclical regulatory patterns are consistent with polyvagal theory's hierarchical model of autonomic response and with attachment-based models of affect regulation (Porges, 2011; Schoore, 2003). Trauma research further demonstrates that psychological stability improves when individuals develop regulatory capacity rather than relying solely on symptom suppression (van der Kolk, 2014).

By providing a structured visual representation of these processes, the model supports recognition of:



- The physiological origins of emotional responses
- Embodied storage of traumatic memory
- Cognitive perceptual shaping through belief systems
- The restorative role of co-regulation and environmental safety

Such psychoeducational translation is widely recognised as central to recovery oriented mental health practice (Davidson et al., 2006). Integration with Trauma Informed and Recovery Oriented Care



## Where My Work Integrates with Duchess Dr. Letitia's Doctoral Research:

Duchess Dr. Letitia's doctoral framework demonstrates that sustainable mental health emerges through:

- Voluntary participation in treatment
- Collaborative and interdisciplinary care
- Cultural responsiveness
- Personal agency and autonomy
- Structured daily functioning

These principles are foundational to recovery-oriented mental health systems (Slade et al., 2014). Psychoeducational and self-regulation frameworks have been shown to enhance treatment engagement, emotional awareness, and functional outcomes by increasing perceived agency and internal safety (Mead & MacNeil, 2006).

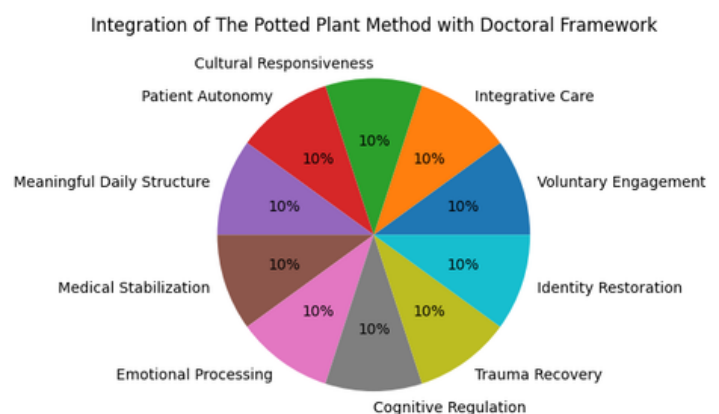
Within this context, the Potted Plant Method functions as a regulatory and psychoeducational bridge connecting:

- Medical stabilisation

- Emotional processing
- Cognitive restructuring
- Trauma integration
- Identity development

This integrative orientation reflects contemporary shifts toward person centred and trauma responsive systems of care (SAMHSA, 2014).

*In clinical language:* My framework operates as a psycho-educational, somatic, and attachment-informed regulation system that enables individuals to maintain stability between medical and therapeutic interventions.



## Why This Matters for Higher Learning and Mental Health Practice:

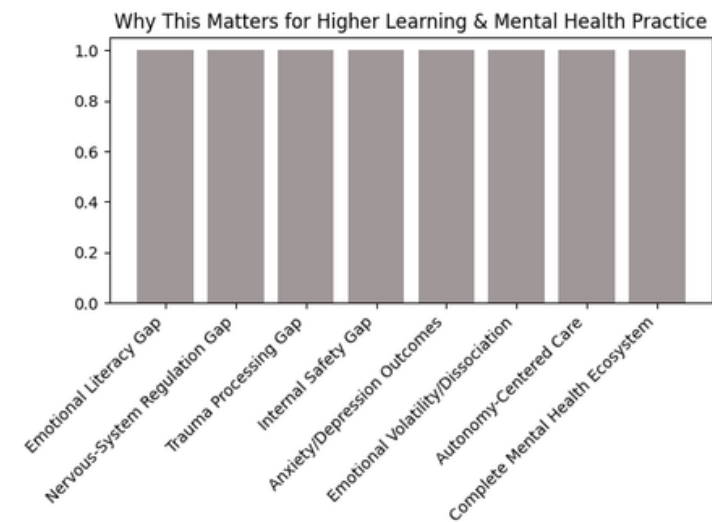
Large segments of the population have never been taught:

- Emotional literacy
- Nervous-system regulation
- Trauma processing
- Internal safety

Many people have developed survival strategies that later appear as anxiety, depression, emotional instability, or dissociation. The Potted Plant Method brings these hidden processes into the open and makes them teachable, helping individuals engage in the autonomy centered, integrative care supported by Duchess Dr. Letitia's research. We are not facing a worldwide crisis of broken minds. We are facing a global lack of emotional and nervous system education. By bringing together Duchess Dr. Letitia's clinical and interdisciplinary scholarship with my lived experience based neuro emotional mapping,



We present a complete mental health ecosystem one that respects both medical necessity and the realities of human development.



## APPENDIX A:

The Potted Plant Method as a Neuro Emotional Regulation and Recovery Framework

### A1. Purpose of Inclusion:

This appendix presents the Potted Plant Method as a lived experience derived psycho educational & neuro emotional regulation framework designed to complement the trauma-informed, voluntary, and integrative mental health model articulated by Dr. Duchess Letitia Kapuscinska. Where Duchess Dr. Letitia's doctoral research establishes the clinical, ethical, & interdisciplinary foundations of recovery oriented mental health care, this appendix provides a practical and embodied translation of those principles into a form that individuals can understand, internalize, and apply in daily life. The model addresses a critical gap in contemporary mental health systems: the lack of accessible frameworks for understanding emotional turbulence, nervous-system dysregulation, and trauma conditioned behavioral patterns.

### Conflict of interest:

Author declares there is no conflict of interest.

## References :

1. Anthony, W. A. (1993). Recovery from mental illness: The guiding vision of the mental health service system in the 1990s. *Psychosocial Rehabilitation Journal*, 16(4), 11–23.
2. Bernal, G., Jiménez-Chafey, M. I., & Domenech Rodríguez, M. M. (2009). Cultural adaptation of treatments: A resource for considering culture in evidence based practice. *Professional Psychology: Research and Practice*, 40(4), 361–368.
3. Browne, A. J., Varcoe, C., Ford-Gilboe, M., & Wathen, C. N. (2016). EQUIP healthcare: An overview of a multi-component intervention to enhance equity-oriented care in primary health care settings. *International Journal for Equity in Health*, 15, Article 87.
4. Davidson, L., Chinman, M., Sells, D., & Rowe, M. (2006). Peer support among adults with serious mental illness: A report from the field. *Schizophrenia Bulletin*, 32(3), 443–450.
5. Fallot, R. D., & Harris, M. (2009). Creating cultures of trauma informed care (CCTIC): A self assessment and planning protocol. *Community Connections*.
6. Gone, J. P. (2009). A community based treatment for Native American historical trauma: Prospects for evidence based practice. *Spirituality in Clinical Practice*, 1(2), 250–266.
7. Gone, J. P. (2013). Redressing First Nations historical trauma: Theorizing mechanisms for Indigenous culture as mental health treatment. *Transcultural Psychiatry*, 50(5), 683
8. Gone, J. P., & Kirmayer, L. J. (2020). Advancing Indigenous mental health research: Ethical, conceptual, and methodological challenges. *Transcultural Psychiatry*, 57(2), 235–249.
9. Herman, J. L. (1997). *Trauma and recovery: The aftermath of violence from domestic abuse to political terror*. Basic Books.
10. Isobel, S., & Edwards, C. (2015). Using trauma informed care as a framework to promote therapeutic alliances in mental health nursing. *Issues in Mental Health Nursing*,
11. Kapuscinska, A. (2024). *Restored: A lived experience framework for integrative mental health recovery*. [Unpublished practitioner monograph reflective framework].
12. Kirmayer, L. J., Dandeneau, S., Marshall, E., Phillips, M. K., & Williamson, K. J. (2011). Rethinking resilience from Indigenous perspectives. *Canadian Journal of Psychiatry*, 56(2), 84–91.

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13. Kirmayer, L. J. (2012). Rethinking cultural competence. *Transcultural Psychiatry*, 49(2), 149–164.
  14. Kleinman, A. (1988). *Rethinking psychiatry: From cultural category to personal experience*. Free Press.
  15. Kübler-Ross, E. (1969). *On death and dying*. Macmillan.
  16. Leamy, M., Bird, V., Le Boutillier, C., Williams, J., & Slade, M. (2011). Conceptual framework for personal recovery in mental health: Systematic review and narrative synthesis. *British Journal of Psychiatry*, 199(6), 445–452.
  17. Mead, S., & MacNeil, C. (2006). Peer support: What makes it unique? *International Journal of Psychosocial Rehabilitation*, 10(2), 29–37.
  18. Menschner, C., & Maul, A. (2016). Key ingredients for successful trauma-informed care implementation. Center for Health Care Strategies.
  19. Repper, J., & Carter, T. (2011). A review of the literature on peer support in mental health services. *Journal of Mental Health*, 20(4), 392–411.
  20. Ridgway, P. (2001). Re-storying psychiatric disability: Learning from first-person recovery narratives. *Psychiatric Rehabilitation Journal*, 24(4), 335–343.
  21. Slade, M., Amering, M., & Oades, L. (2014). Recovery: An international perspective. *Epidemiology and Psychiatric Sciences*, 17(2), 128–137.
  22. Substance Abuse & Mental Health Services Administration. (2014). SAMHSA's concept of trauma and guidance for a trauma-informed approach (HHS Publication No. SMA 14-4884). U.S. Department of Health and Human Services.
  23. Sue, D. W. (2006). *Multicultural social work practice*. John Wiley & Sons.
  24. Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2012). Racial microaggressions in everyday life: Implications for clinical practice. *American Psychologist*, 62(4), 271–286.
  25. Topor, A., Borg, M., Di Girolamo, S., & Davidson, L. (2011). Not just an individual journey: Social aspects of recovery. *International Journal of Social Psychiatry*, 57(1), 90–99.
  26. Tseng, W. S. (2004). Culture and psychotherapy: Asian perspectives. *Journal of Mental Health*, 13(2), 151–161.
  27. Williams, D. R., & Mohammed, S. A. (2009). Discrimination and racial disparities in health: Evidence and needed research. *Journal of Behavioral Medicine*, 32(1), 20–47.